

RHEAP

Rules and Regulations

of the

Remote Health Examination Association Pakistan (RHEAP)

An independent, non-profit, non-political and non-religious association advancing research, education and the safe, evidence-based use of Remote Health Examination in Pakistan.

Adopted at the Founding Meeting of RHEAP. Where any other RHEAP material conflicts with this document, this document prevails.

1. Name

The name of the Association shall be Remote Health Examination Association Pakistan, abbreviated as RHEAP.

2. Nature of the Association

RHEAP is an independent, non-profit, non-political and non-religious association established to advance research, education, professional cooperation and responsible implementation of Remote Health Examination in Pakistan.

RHEAP shall operate independently from individual healthcare companies, healthcare providers, technology companies, donors and other commercial organisations.

The Association shall not operate for the commercial benefit of any individual company, product, technology or service.

No member, donor, sponsor, company or other organisation shall acquire ownership, voting rights, governance rights or control over RHEAP through donations, sponsorship, cooperation or other financial support.

3. Purpose

The purpose of RHEAP is to research, promote and support the use of Remote Health Examination in Pakistan with the primary objective of achieving better patient outcomes.

The Association seeks to study and demonstrate how Remote Health Examination can improve access to healthcare, strengthen clinical assessment, support earlier identification of health conditions, improve continuity of care and make healthcare professionals and specialist expertise more accessible to patients regardless of geographical location.

RHEAP shall promote clinically appropriate, safe, ethical and evidence-based use of Remote Health Examination.

Particular attention shall be given to communities and patient groups where access to physicians, specialists, diagnostics, regular monitoring or healthcare facilities is limited.

RHEAP shall bring together healthcare professionals, researchers, universities, healthcare organisations, students, technology professionals, policymakers and other individuals interested in Remote Health Examination and improving patient outcomes in Pakistan.

4. Initial Health Focus Areas

RHEAP may address any health condition or healthcare challenge where Remote Health Examination can contribute to better patient outcomes.

During its initial phase, the Association shall particularly focus on four areas of major importance to healthcare in Pakistan.

4.1 Women's Health — RHEAP shall promote research, education and practical applications of Remote Health Examination that improve women's access to healthcare, clinical assessment, screening, monitoring

and specialist expertise. The area may include maternal health, reproductive health, pregnancy related care, gynaecological health, cervical health and other areas relevant to improving women's health outcomes.

4.2 Hypertension — RHEAP shall support approaches that improve the identification, clinical assessment, monitoring and management of hypertension. The Association shall particularly explore how Remote Health Examination can improve regular measurement, follow-up and access to healthcare professionals for patients who otherwise have limited access to healthcare services.

4.3 Diabetes — RHEAP shall promote the use of Remote Health Examination in the prevention, identification, monitoring and long-term management of diabetes. The Association may support models that improve regular follow-up, collection of clinically relevant measurements, patient education and access to healthcare professionals.

4.4 Tuberculosis and Respiratory Health — RHEAP shall promote research and practical approaches in which Remote Health Examination can support clinical assessment, screening pathways, referral, monitoring and follow-up related to tuberculosis and other respiratory conditions. Remote Health Examination shall complement, not replace, laboratory testing, imaging or other diagnostic procedures where these are clinically required.

These four focus areas shall not limit the future activities of RHEAP. The Association may establish additional health focus areas according to healthcare needs in Pakistan.

5. Definition of Remote Health Examination

For the purposes of RHEAP, Remote Health Examination, or RHE, refers to the use of digital technologies that enable a healthcare professional to examine, clinically assess or obtain clinically relevant information from a patient when the responsible healthcare professional is geographically separated from the patient.

Remote Health Examination may include: real-time audio and video communication; connected medical devices and diagnostic instruments; real-time transmission of clinical measurements, images, sounds and other examination data; real-time guidance of a healthcare professional or appropriately trained person physically present with the patient; digital clinical decision support; artificial intelligence where appropriate; and integration with electronic health records and other digital healthcare systems.

RHEAP shall maintain a technology-neutral approach. Remote Health Examination shall not be defined by reference to any particular commercial product, manufacturer, platform or service provider.

6. Objectives and Activities

To achieve its purpose, RHEAP may: conduct and support research related to Remote Health Examination; support clinical studies, evaluations, surveys and pilot projects; collect and publish evidence concerning patient outcomes, healthcare access, clinical quality and healthcare delivery; organise conferences, seminars, workshops, webinars and professional meetings; provide and support professional education and training; develop recommendations, frameworks and best practices; establish scientific, clinical and technical working groups; cooperate with hospitals, universities, healthcare professionals and research institutions; cooperate with governmental authorities and public healthcare organisations; cooperate with NGOs and international organisations; facilitate national and international exchange of knowledge; increase professional and public awareness of Remote Health Examination; support academic research and

publication; promote patient safety, clinical quality and ethical use of Remote Health Examination; contribute to professional discussion concerning standards, interoperability and regulation; and undertake other lawful non-profit activities supporting the objectives of RHEAP.

RHEAP is not a healthcare regulator, medical licensing authority or medical device regulator and shall not represent itself as such.

7. Membership

Membership of RHEAP shall be open to individuals who support the purpose and objectives of the Association.

Membership may include healthcare professionals as well as other individuals with a genuine interest in Remote Health Examination.

Members may therefore include physicians, nurses, midwives, allied healthcare professionals, students, researchers, academics, healthcare administrators, public health professionals, technology professionals, engineers, entrepreneurs and other interested individuals.

Employment in healthcare shall not be a requirement for membership.

Membership shall always be held by an individual in his or her personal capacity.

Companies, hospitals, universities, NGOs and other legal entities shall not be voting members of RHEAP.

8. Admission of Members

Applications for membership shall be submitted according to procedures established by the Executive Committee.

Membership shall become effective following approval of the application and registration of the individual in the membership register.

Applications should normally be accepted where the applicant supports the purpose and principles of RHEAP.

The Executive Committee may reject an application where there are reasonable grounds to believe that membership would materially conflict with the purpose, independence, ethical principles or reputation of RHEAP.

9. Rights of Members

Each member in good standing shall have the right to participate in RHEAP activities, attend General Meetings, vote at General Meetings, stand for election to the Executive Committee, participate in committees and working groups and submit proposals to the Association.

Each voting member shall have one vote.

10. Responsibilities of Members

Members shall support the objectives of RHEAP, comply with these Rules, respect decisions lawfully made by the Association, act in accordance with professional integrity and patient safety and disclose relevant conflicts of interest where appropriate.

Members shall pay any membership fee established by the General Meeting.

11. Membership Fees

The General Meeting may establish an annual membership fee.

The Association may establish reduced or free membership for students or other defined groups.

The General Meeting may also decide that membership shall be free of charge.

12. Termination of Membership

Membership shall terminate upon resignation, death, failure to fulfil membership obligations or serious violation of these Rules.

A member may also be expelled if his or her conduct materially harms the Association or is fundamentally inconsistent with its objectives or ethical principles.

Before expulsion, the member shall be given a reasonable opportunity to provide an explanation.

13. General Meeting

The General Meeting is the highest decision-making body of RHEAP.

An Annual General Meeting shall be held once every calendar year.

The Annual General Meeting shall review the activities of RHEAP, approve the annual report and financial statements, approve the activity plan and budget, determine membership fees where applicable, elect members of the Executive Committee and decide other matters properly submitted to the meeting.

14. Extraordinary General Meeting

An Extraordinary General Meeting may be called by the Executive Committee.

An Extraordinary General Meeting shall also be called where at least twenty percent of voting members submit a written request stating the matter to be considered.

15. Notice and Participation

Members shall normally receive at least fourteen days' notice of a General Meeting.

Meetings may be organised physically, electronically or in hybrid form where permitted by applicable law.

16. Voting

Unless otherwise provided in these Rules or required by applicable law, decisions shall be made by a simple majority of votes cast.

Each voting member shall have one vote.

Amendments to these Rules shall require a two-thirds majority of votes cast.

17. Executive Committee

The affairs of RHEAP shall be managed by an Executive Committee consisting of five to nine members.

The Executive Committee shall include at least: President; Vice President; General Secretary; Treasurer; Executive Committee Member or Members.

Members of the Executive Committee shall be elected by the General Meeting.

The normal term of office shall be two years. Members may be re-elected.

18. Responsibilities of the Executive Committee

The Executive Committee shall manage the activities and finances of RHEAP, implement decisions of the General Meeting, prepare annual plans and budgets, maintain the membership register, prepare annual reports and financial statements, establish committees and working groups, approve cooperation arrangements and ensure compliance with applicable laws and regulations.

The Executive Committee shall also safeguard the professional and commercial independence of the Association.

19. Officers

The President shall lead the Association, chair meetings and represent RHEAP in official and professional matters.

The Vice President shall support the President and perform the duties of the President when required.

The General Secretary shall be responsible for administration, records, meeting documentation, notices and the membership register.

The Treasurer shall oversee financial administration, accounting, financial reporting and banking arrangements.

20. Scientific and Professional Committees

The Executive Committee may establish permanent or temporary committees and working groups.

Initial committees may include: Women's Health Committee; Hypertension and Cardiovascular Health Committee; Diabetes Committee; Tuberculosis and Respiratory Health Committee; Clinical and Scientific Committee; Research Committee; Education and Training Committee; Technology and Standards Committee; and Ethics and Patient Safety Committee.

RHEAP may invite external experts to participate in committees and working groups.

21. Independence and Technology Neutrality

RHEAP shall operate independently of commercial interests.

The Association may cooperate with healthcare companies, technology providers, hospitals and other commercial organisations where such cooperation supports the objectives of RHEAP.

No commercial organisation shall have the right to control the governance, research agenda, scientific positions, clinical recommendations or professional activities of RHEAP.

RHEAP shall remain technology neutral.

Research, education and professional recommendations shall be based on patient outcomes, clinical evidence, patient safety, clinical quality, accessibility, usability and other appropriate professional considerations.

22. Conflict of Interest

Members of the Executive Committee and persons participating in scientific or professional decision-making shall disclose relevant financial, professional or other material conflicts of interest.

A person with a material conflict of interest shall not participate in a decision where that conflict could materially affect the independence of the decision.

23. Supporting Organisations, Partners and Sponsors

Companies, hospitals, universities, NGOs, foundations, professional organisations and other organisations may support or cooperate with RHEAP.

Such organisations may be recognised as Supporting Organisations, Partners or Sponsors.

Such status shall not provide voting rights, ownership, governance rights or automatic representation on the Executive Committee.

Financial support shall not give the supporting organisation control over RHEAP's scientific, clinical or professional positions.

24. Financial Resources

The financial resources of RHEAP may include membership fees, donations, grants, research funding, sponsorship, conference and event income, educational programme income and other lawful sources consistent with the non-profit purpose of the Association.

All income and assets of RHEAP shall be used exclusively to advance the objectives of the Association.

No profits or assets shall be distributed to members.

25. Donations

RHEAP may receive financial or in-kind donations from individuals, companies, foundations, governmental bodies, international organisations and other lawful sources.

A donation shall not provide the donor with ownership, voting rights, governance rights or control over RHEAP.

Material financial support shall be transparently disclosed where appropriate.

26. Financial Administration

Funds belonging to RHEAP shall be maintained in an account held in the name of the Association.

Proper accounting records shall be maintained.

The Executive Committee shall establish appropriate financial controls.

Significant payments and financial commitments shall normally require the approval of at least two authorised officers.

27. Financial Reporting

Annual financial statements shall be prepared and presented to the Annual General Meeting.

The accounts shall be audited or independently reviewed where required by applicable law or decided by the General Meeting.

28. Compensation and Expenses

Members of the Executive Committee shall not receive remuneration solely for serving on the Executive Committee.

Reasonable expenses incurred in approved RHEAP activities may be reimbursed.

RHEAP may employ staff, commission research or purchase professional services where necessary.

29. Political and Religious Independence

RHEAP shall remain politically and religiously independent.

The Association may cooperate with ministries, governmental authorities, policymakers and other public bodies on matters related to healthcare, research and Remote Health Examination.

30. Name and Logo

The name Remote Health Examination Association Pakistan, abbreviation RHEAP, logo and official publications shall be controlled by the Association.

No member, company or other organisation may use the RHEAP name or logo in a manner suggesting official endorsement, certification or partnership without authorisation from RHEAP.

31. Amendments

These Rules may be amended by the General Meeting with a two-thirds majority of votes cast, subject to applicable Pakistani law.

32. Dissolution

RHEAP may be dissolved in accordance with applicable Pakistani law.

Following settlement of all debts and liabilities, remaining assets shall not be distributed among members.

Remaining assets shall be transferred to another non-profit organisation with healthcare, scientific, educational or similar public-interest objectives.

33. Applicable Law

RHEAP shall operate in accordance with the applicable laws and regulations of Pakistan and the province in which the Association is registered.

Where any provision of these Rules conflicts with mandatory law, applicable law shall prevail.

34. Founding Executive Committee

The founding members shall elect the first Executive Committee at the Founding Meeting of RHEAP.

The first Executive Committee shall serve until the first Annual General Meeting unless otherwise determined in accordance with these Rules.

35. Adoption

These Rules and Regulations were adopted at the Founding Meeting of Remote Health Examination Association Pakistan (RHEAP).